

TRAPS

- >7 DAYS
- WEEKS
- WEEKS
- PERIORBITAL EDEMA
- MIGRATORY MYALGIA
- AD
- AD
- TNFRSF1A
- MIGRATORY MYALGIA
- LEFT CALF MUSCLE BRIGHT RED w/ OVERLYING TENDER ERYTHEMA
- COLCH
- 15% AMYLOID
- CYSTINE mutations = WORST
- BEST
- INFLIX
- CANAKINUMAB
- BEST
- CERVICAL LAD
- INFECTION
- VACCINE
- INFECTION
- STRESS

HIDS

- APHTHA
- DIFFUSE MACULOPAPULAR RASH
- COLD
- AMYLOID 25%
- CANAKINUMAB
- MEVALONATE ↑
- 3-7 DAYS RARE
- PALMS + SOLES

MWS

- FCAS
- URTL RASH
- NLRP3
- RILONACEPT
- ANAKINRA
- IL-1 = LIFE-CHANGING
- HEARING LOSS
- NOMID
- ASEPTIC MENINGITIS
- UVEITIS → BLIND
- MOSAIC 1/3
- NEONATAL
- ABORTS ATTACK
- may ↓ interval
- 3-5 DAYS
- MOST COMMON IN CHILDREN
- NON-MENDELIAN

PFAPA

- CLOCK-LIKE 3-8 WK
- NO RASH
- NO ARTHRITIS
- NO SEROSITIS
- NO AMYLOID
- ADENITIS
- PHARYNGITIS
- TONSILLECTOMY can CURE
- MOST COMMON IN CHILDREN

ATTACK DURATION

- FMF: 24-72h
- HIDS: 3-7d
- PFAPA: 3-5d
- TRAPS: >7d-WEEKS

TRAPS

- ONLY FOR FMF
- ONLY FOR UNUSED FMF
- ONLY FOR FMF

1. Attack duration clock

WHAT YOU SEE

Clocks: FMF 24–72h, HIDS 3–7d, PFAPA 3–5d

WHAT IT ENCODES

Attack length is the single best bedside discriminator among the periodic fevers: FMF is shortest at 1–3 days (12–72h), HIDS runs 3–7 days, PFAPA 3–5 days at clockwork intervals, and TRAPS is the longest at >7 days to several weeks. Pair duration with the trigger and ancestry to nail the syndrome.

BOARD TRAP

If a stem emphasizes a very LONG (>1 week) attack, the answer is TRAPS, not FMF. If it stresses a vaccine-triggered 3–7 day attack in an infant, think HIDS. Anchoring only on 'recurrent fever' without using duration is the trap.

2. TRAPS — TNFRSF1A

WHAT YOU SEE

Long-bearded man, TNFRSF1A, migratory myalgia

WHAT IT ENCODES

TRAPS (TNF-receptor-associated periodic syndrome) is autosomal DOMINANT due to TNFRSF1A (p55 TNF receptor) mutation causing defective receptor shedding and intracellular receptor retention. Attacks last >7 days (often 1–3 weeks) with migratory myalgia, periorbital edema, overlying tender erythema, conjunctivitis, and abdominal pain. AA amyloidosis is a long-term risk.

BOARD TRAP

TRAPS is the autosomal DOMINANT periodic fever and the one with very long (>1 week) attacks — the deliberate contrast to recessive, short-attack FMF. Anti-TNF (etanercept) or IL-1 blockade is used; colchicine does NOT work for TRAPS.

3. TRAPS periorbital edema

WHAT YOU SEE

PERIORBITAL EDEMA + migratory myalgia tags

WHAT IT ENCODES

Periorbital edema with conjunctivitis plus migratory myalgia and an overlying warm tender erythematous patch (centrifugal, moving distally over a limb) is the TRAPS signature physical exam. These features track with the prolonged attacks.

BOARD TRAP

Periorbital edema + migratory myalgia = TRAPS, not FMF (FMF has erysipeloid rash on the ankle/foot, not periorbital edema). Forgetting that TRAPS can ALSO cause AA amyloidosis (like FMF) is a second trap — both warrant SAA monitoring.

4. HIDS / MVK

WHAT YOU SEE

Crying baby, MEVALONATE↓, mevalonate kinase deficiency

WHAT IT ENCODES

HIDS (hyper-IgD syndrome) is autosomal recessive mevalonate kinase (MVK) deficiency, an enzyme in the cholesterol/isoprenoid pathway; complete deficiency causes severe mevalonic aciduria. It presents in infancy, classically with attacks triggered by childhood VACCINATIONS, infection, or stress. Labs show elevated IgD (and often IgA) and elevated urinary mevalonic acid during attacks.

BOARD TRAP

HIDS = mevalonate kinase deficiency with vaccine-triggered attacks in an infant — do NOT pick NLRP3 (cryopyrinopathy) or pyrin/MEFV (FMF). Note elevated IgD is suggestive but neither sensitive nor specific; urinary mevalonic acid and MVK genetics confirm.

5. HIDS features

WHAT YOU SEE

Cervical lymphadenopathy, diffuse rash, aphthae

WHAT IT ENCODES

HIDS attacks last 3–7 days with high fever plus prominent tender CERVICAL lymphadenopathy, abdominal pain/diarrhea/vomiting, aphthous and genital ulcers, a diffuse maculopapular rash, arthralgia/arthritis, and splenomegaly. Amyloidosis is rare in HIDS compared with FMF/TRAPS.

BOARD TRAP

Prominent cervical lymphadenopathy + vaccine trigger + infantile onset points to HIDS over FMF. The discriminator from FMF: FMF rarely has lymphadenopathy and is shorter; HIDS rarely progresses to amyloidosis.

6. FCAS — cold trigger

WHAT YOU SEE

Shivering child, snowflake, NLRP3, 'cold' tag

WHAT IT ENCODES

FCAS (familial cold autoinflammatory syndrome) is the mildest cryopyrinopathy, autosomal dominant NLRP3 (cryopyrin) gain-of-function. Generalized COLD exposure triggers urticarial rash, fever, arthralgia, and conjunctivitis within 1–2 hours, lasting <24h. It is IL-1β driven.

BOARD TRAP

FCAS, MWS, and NOMID are all NLRP3/cryopyrinopathies on one severity spectrum (FCAS mildest → NOMID most severe) — do not split them across different genes. Cold-induced urticaria + fever = FCAS, not cold urticaria alone (which lacks systemic fever and NLRP3).

7. MWS — hearing loss

WHAT YOU SEE

Child with HEARING LOSS, NLRP3

WHAT IT ENCODES

Muckle-Wells syndrome is the intermediate-severity NLRP3 cryopyrinopathy: recurrent urticaria, fever, arthralgia, conjunctivitis, and progressive sensorineural HEARING LOSS, with a significant risk of AA amyloidosis. IL-1 inhibition can halt progression including hearing decline.

BOARD TRAP

Progressive sensorineural hearing loss in a cryopyrinopathy = MWS (sits between FCAS and NOMID). Don't attribute hearing loss to FMF; and remember MWS, unlike FCAS, carries real amyloidosis risk.

8. NOMID / CINCA

WHAT YOU SEE

Infant, NLRP3, neonatal/aseptic meningitis tags

WHAT IT ENCODES

NOMID (neonatal-onset multisystem inflammatory disease), also called CINCA, is the most severe NLRP3 cryopyrinopathy: NEONATAL onset with chronic aseptic meningitis, papilledema/vision loss, sensorineural hearing loss, deforming arthropathy (overgrowth of patella/long bones), urticarial rash, and developmental delay. Often de novo somatic/germline NLRP3 mutations.

BOARD TRAP

NOMID is the severe end of the NLRP3 spectrum — neonatal onset + chronic aseptic meningitis + deforming arthropathy. The trap is calling the meningitis infectious: CSF is sterile (aseptic), and the treatment is IL-1 blockade, not antibiotics.

9. IL-1 = life-changing

WHAT YOU SEE

'IL-1 LIFE-CHANGING' caption under cryopyrinopathy kids

WHAT IT ENCODES

All three cryopyrinopathies (FCAS/MWS/NOMID) are driven by NLRP3-mediated overproduction of IL-1 β and respond dramatically to IL-1 inhibitors — anakinra (daily SC IL-1Ra) or canakinumab (anti-IL-1 β monoclonal); rilonacept is another option. Early IL-1 blockade can prevent hearing loss, vision loss, and amyloidosis.

BOARD TRAP

The treatment for NLRP3 syndromes is IL-1 blockade, NOT colchicine — colchicine is FMF's drug and does not control cryopyrinopathies. Picking colchicine for a cold/hearing-loss/neonatal-meningitis stem is the wrong answer.

10. PFAPA

WHAT YOU SEE

Boy with red cheeks, PFAPA, 'most common in children'

WHAT IT ENCODES

PFAPA = Periodic Fever, Aphthous stomatitis, Pharyngitis, cervical Adenitis. It is the MOST COMMON periodic fever syndrome of childhood, onset typically before age 5, and is non-Mendelian/non-genetic (no single causative gene). Between episodes children are entirely well with normal growth and development.

BOARD TRAP

PFAPA is the most common pediatric periodic fever and is NON-genetic — do not order/expect a single-gene test. The pharyngitis can be mistaken for recurrent strep, but throat cultures are negative and antibiotics don't help.

11. PFAPA clockwork + no amyloid

WHAT YOU SEE

Clocklike interval, NO RASH, NO AMYLOID, normal growth

WHAT IT ENCODES

PFAPA episodes recur at strikingly regular ('clockwork') intervals (roughly every 3–6 weeks), each lasting 3–5 days, WITHOUT rash, serositis, or significant arthritis, with preserved normal growth. Crucially, PFAPA does NOT cause amyloidosis and tends to self-resolve by adolescence.

BOARD TRAP

PFAPA does NOT progress to AA amyloidosis (unlike FMF, TRAPS, and MWS) and is reassuringly benign with clockwork periodicity and normal growth. Calling it a serious amyloid risk, or attributing rash/serositis to it, is the trap.

12. PFAPA — steroids / tonsillectomy

WHAT YOU SEE

'Aborts attack' single steroid dose; tonsillectomy can cure

WHAT IT ENCODES

A SINGLE dose of corticosteroid (e.g., prednisone/prednisolone ~1–2 mg/kg) reliably aborts an individual PFAPA attack, often within hours. Tonsillectomy can be curative in selected children; colchicine or cimetidine may reduce episode frequency. The disorder typically resolves spontaneously with age.

BOARD TRAP

One steroid dose aborts a PFAPA attack — this is the opposite of FMF, where corticosteroids do NOT abort or prevent attacks (colchicine prophylaxis does). Mixing up the steroid responsiveness is a classic distractor.

13. Colchicine for FMF only

WHAT YOU SEE

COLCH labels and pills at the FMF/HIDS seats

WHAT IT ENCODES

Colchicine is specifically the FMF drug (preventing attacks and amyloidosis). It has limited or no role in the IL-1-driven cryopyrinopathies (FCAS/MWS/NOMID), in TRAPS, or in HIDS; it may modestly reduce PFAPA frequency but is not the abortive agent. Match the mechanism to the drug.

BOARD TRAP

Do NOT apply colchicine to every periodic fever — it is the answer for FMF, while IL-1 blockade treats the cryopyrinopathies (and refractory FMF), and TRAPS responds to anti-TNF/IL-1. Choosing colchicine for an NLRP3 or TRAPS stem is the wrong-drug trap.